

Documentation of Current Medications in the Medical Record (Measure)

Definition:	Percentage of visits for patients aged 18 years and older for which the eligible professional or eligible clinician attests to documenting a list of current medications using all immediate resources available on the date of the encounter. This list must include ALL known prescriptions, over-the-counters, herbals, and vitamin/mineral/dietary (nutritional) supplements AND must contain the medications' name, dosage, frequency and route of administration.
Improvement Notation:	Higher score indicates better quality
Measurement Period:	January 1, 20XX through December 31, 20XX
Transmission Format:	TBD
Scoring:	Proportion
Type:	PROCESS
ID:	68
Version:	7
NQF:	0419
GUID:	9a032d9c-3d9b-11e1-8634-00237d5bf174
Measure Set:	CLINICAL QUALITY MEASURE SET
Eligibility:	Eligible Professionals / Eligible Clinicians
Domain:	Patient Safety

Population Criteria Section

Initial Population	AND: Age >= 18 year(s) at: "Measurement Period"
	AND: "Occurrence A of Encounter, Performed: Medications Encounter Code Set" during "Measurement Period"
Denominator	AND: Initial Population
Denominator Exclusions	None
Numerator	AND: "Procedure, Performed: Current Medications Documented SNMD" during "Occurrence A of Encounter, Performed: Medications Encounter Code Set"
Numerator Exclusions	None
Denominator Exceptions	OR: "Procedure, Performed not done: Medical or Other reason not done" for "Current Medications Documented SNMD" during "Occurrence A of Encounter, Performed: Medications Encounter Code Set"
Stratification	None

Data Criteria (QDM Variables)

Data Criteria (QDM Variables)	None
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Value Set Disclaimer:	Access to the Value Set Authority Center requires a free Unified Medical Language System® Metathesaurus License. It is expected that any use of value sets is consistent with these licensing requirements and copyright protections. To access the 2014 CQMs' value sets in USHIK, you need to have a free Unified Medical Language System® Metathesaurus License and abide by its conditions.
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Data Criteria

Category	Data Element	Value Set	
Encounter	Encounter, Performed: Medications Encounter Code Set	Value Set Name	Medications Encounter Code Set
		Value Set OID	2.16.840.1.113883.3.600.1.1834
		Value Set Version	eCQM Update 2017-05-05
		HCPCS	G0101, G0108, G0270, G0402, G0438, G0439
		CPT	59400, 59510, 59610, 59618, 90791, 90792, 90832, 90834, 90837, 90839, 92002, 92004, 92012, 92014, 92507, 92508, 92526, 92537, 92538, 92540, 92541, 92542, 92544, 92545, 92547, 92548, 92550, 92557, 92567, 92568, 92570, 92585, 92588, 92626, 96116, 96150, 96151, 96152, 97001, 97002, 97003, 97004, 97532, 97802, 97803, 97804, 98960, 98961, 98962, 99024, 99201, 99202, 99203, 99204, 99205, 99212, 99213, 99214, 99215, 99221, 99222, 99223, 99281, 99282, 99283, 99284, 99285, 99324, 99325, 99326, 99327, 99328, 99334, 99335, 99336, 99337, 99341, 99342, 99343, 99344, 99345, 99347, 99348, 99349, 99350, 99385, 99386, 99387, 99395, 99396, 99397, 99495, 99496
		SNOMEDCT	10197000, 108220007, 108221006, 108224003, 108311000, 13607009, 14736009, 165171009, 18091003, 18512000, 185349003, 185463005, 185465003, 209099002, 210098006, 225967005, 252592009, 252624005, 270427003, 270430005, 273643004, 274803000, 277404009, 284015009, 30346009, 308335008, 32537008, 34651001, 35025007, 36228007, 370803007, 37894004, 385973000, 386372009, 390906007, 405096004, 406547006, 408983003, 410155007, 410157004, 410158009, 410160006, 410170008, 439708006, 439952009, 440524004, 46662001, 48423005, 50357006, 53555003, 54290001, 63547008, 66902005, 78318003, 83607001, 8411005, 86013001, 90526000, 91573000

Procedure	Procedure, Performed not done: Medical or Other reason not done	Value Set Name	Medical or Other reason not done
		Value Set OID	2.16.840.1.113883.3.600.1.1502
		Value Set Version	eCQM Update 2017-05-05
		SNOMEDCT	183932001, 397745006, 407563006, 428119001
Procedure	Procedure, Performed: Current Medications Documented SNMD	Value Set Name	Current Medications Documented SNMD
		Value Set OID	2.16.840.1.113883.3.600.1.462
		Value Set Version	eCQM Update 2017-05-05
		SNOMEDCT	428191000124101

Supplemental Data Elements

Category	Data Element	Value Set	
Characteristics (patient or provider)	Patient Characteristic Ethnicity: Ethnicity	Value Set Name	Ethnicity
		Value Set OID	2.16.840.1.114222.4.11.837
		Value Set Version	eCQM Update 2017-05-05
		CDCREC	2135-2, 2186-5
Characteristics (patient or provider)	Patient Characteristic Payer: Payer	Value Set Name	Payer
		Value Set OID	2.16.840.1.114222.4.11.3591
		Value Set Version	eCQM Update 2017-05-05
		SOP	1, 11, 111, 112, 113, 119, 12, 121, 122, 123, 129, 13, 14, 19, 191, 2, 21, 211, 212, 213, 219, 22, 23, 24, 25, 26, 29, 291, 3, 31, 311, 3111, 3112, 3113, 3114, 3115, 3116, 3119, 312, 3121, 3122, 3123, 313, 32, 321, 3211, 3212, 32121, 32122, 32123, 32124, 32125, 32126, 322, 3221, 3222, 3223, 3229, 33, 331, 332, 333, 334, 34, 341, 342, 343, 349, 35, 36, 361, 362, 369, 37, 371, 3711, 3712, 3713, 372, 379, 38, 381, 3811, 3812, 3813, 3819, 382, 389, 39, 4, 41, 42, 43, 44, 5, 51, 511, 512, 513, 514, 515, 516, 519, 52, 521, 522, 523, 529, 53, 54, 55, 56, 561, 562, 59, 6, 61, 611, 612, 613, 619, 62, 63, 64, 69, 7, 71, 72, 73, 79, 8, 81, 82, 821, 822, 823, 83, 84, 85, 89, 9, 91, 92, 93, 94, 95, 951, 953, 954, 959, 96, 97, 98, 99, 9999
Characteristics (patient or provider)	Patient Characteristic Race: Race	Value Set Name	Race
		Value Set OID	2.16.840.1.114222.4.11.836
		Value Set Version	eCQM Update 2017-05-05
		CDCREC	1002-5, 2028-9, 2054-5, 2076-8, 2106-3, 2131-1
Characteristics (patient or provider)	Patient Characteristic Sex: ONC Administrative Sex	Value Set Name	ONC Administrative Sex
		Value Set OID	2.16.840.1.113762.1.4.1
		Value Set Version	eCQM Update 2017-05-05
		AdministrativeGender	F, M

Metadata

Measure Steward:	Centers for Medicare & Medicaid Services (CMS)
Measure Developer:	Quality Insights
Endorsed By:	National Quality Forum
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Stratification:	None
Risk Adjustment:	None
Rate Aggregation:	None

Rationale:

Maintaining an accurate and complete medication list has proven to be a challenging documentation endeavor for various health care provider settings. While most of outpatient encounters (2/3) result in providers prescribing at least one medication, hospitals have been the focus of medication safety efforts (Stock et al., 2009). Nassaralla et al. (2007) caution that this is at odds with the current trend, where patients with chronic illnesses are increasingly being treated in the outpatient setting and require careful monitoring of multiple medications. Additionally Nassaralla et al. (2007) reveal that it is in fact in outpatient settings where more fatal adverse drug events (ADE) occur when these are compared to those occurring in hospitals (1 of 131 outpatient deaths compared to 1 in 854 inpatient deaths). In the outpatient setting, adverse drug events (ADEs) occur 25% of the time and over one-third of these are considered preventable (Tache et al., 2011). Particularly vulnerable are patients over 65 years, with evidence suggesting that the rate of ADEs per 10,000 person per year increases with age; 25-44 years old at 1.3; 45-64 at 2.2, and 65 + at 3.8 (Sarkar et al., 2011). Another vulnerable group are chronically ill individuals. These population groups are more likely to experience ADEs and subsequent hospitalization.

A multiplicity of providers and inadequate care coordination among them has been identified as barriers to collecting complete and reliable medication records. Data indicate that reconciliation and documentation continues to be poorly executed with discrepancies occurring in 92% (74 of 80 patients) of medication lists among admittance to the emergency room. Of 80 patients included in the study, the home medications were re ordered for 65% of patients on their admission and of the 65% the majority (29%) had a change in their dosing interval, while 23% had a change in their route of administration, and 13% had a change in dose. A total of 361 medication discrepancies, or the difference between the medications patients were taking before admission and those listed in there admission orders, were identified in at least 74 patients. (Peusschers et al., 2015; Poornima et al., 2015) The study found that "Through an appropriate reconciliation programme, around 80% of errors relating to medication and the potential harm caused by these errors could be reduced." (Peusschers et al., 2015; Poornima et al., 2015)

Documentation of current medications in the medical record facilitates the process of medication review and reconciliation by the provider, which are necessary for reducing ADEs and promoting medication safety. The need for provider to provider coordination regarding medication records, and the existing gap in implementation, is highlighted in the American Medical Association's (AMA) Physician's Role in Medication Reconciliation (2007), which states that "critical patient information, including medical and medication histories, current medications the patient is receiving and taking, and sources of medications, is essential to the delivery of safe medical care. However, interruptions in the continuity of care and information gaps in patient health records are common and significantly affect patient outcomes" (American Medical Association, 2007, pp.7). This is because clinical decisions based on information that is incomplete and/or inaccurate are likely to lead to medication error and ADEs. Weeks et al. (2010) noted similar barriers and identified the utilization of health information technology as an opportunity for facilitating the creation of universal medication lists. One 2015 meta-analysis showed an association between EHR documentation with an overall RR of 0.46 (95% CI = 0.38 to 0.55; $P < 0.001$) and ADEs with an overall RR of 0.66 (95% CI = 0.44 to 0.99; $P = 0.045$). This meta-analysis provides evidence that the use of the EHR can improve the quality of healthcare delivered to patients by reducing medication errors and ADEs (Campanella et al., 2016).

Clinical Recommendation Statement:	The Joint Commission's 2015 Ambulatory Care National Patient Safety Goals guide providers to maintain and communicate accurate patient medication information. Specifically, the section "Use Medicines Safely NPSG.03.06.01" states the following: "Maintain and communicate accurate patient medication information. The types of information that clinicians use to reconcile medications include (among others) medication name, dose, frequency, route, and purpose. Organizations should identify the information that needs to be collected to reconcile current and newly ordered medications and to safely prescribe medications in the future." (Joint Commission, 2015, retrieved at: http://www.jointcommission.org/assets/1/6/2015_NPSG_AHC1.PDF). The National Quality Forum's 2010 update of the Safe Practices for Better Healthcare, states healthcare organizations must develop, reconcile, and communicate an accurate patient medication list throughout the continuum of care (p. 40).
Reference:	American Medical Association (2007). The physician's role in medication reconciliation: Issues, strategies and safety principles. Retrieved from https://bcpsqc.ca/documents/2012/09/AMA-The-physician%e2%80%99s-role-in-Medication-Reconciliation.pdf
Reference:	Campanella Paolo, Lovato Emanuela, Marone Claudio, Fallacara Lucia, Mancuso Agostino, Ricciardi Walter, Specchia Maria Lucia. The impact of electronic health records on healthcare quality: A systematic review and meta-analysis. European journal of public health, 26,1,2016:60-64.
Reference:	Nassaralla, C.L., Naessens, J.M., Chaudhry, R., et al. (2007). Implementation of a medication reconciliation process in an ambulatory internal medicine clinic. Quality and Safety in Health Care 2007; (16), 90-94.
Reference:	National Quality Forum (2010). Safe Practices for Better Healthcare - 2010 Update. Retrieved from http://www.qualityforum.org/Projects/Safe_Practices_2010.aspx
Reference:	Poornima P, Reshma P, Ramakrishnan TV, Rani NV, Devi GS, Shree R, Seshadri P. Medication reconciliation and medication error prevention in an emergency department of a tertiary care hospital. Journal of Young Pharmacists, 2015, 7, 3, 241-249
Reference:	Sarkar, U., Lopez, A., Maselli, J.H., Gonzalez, R. (2011). Adverse Drug Events in U.S. Adult Ambulatory Medical Care. Health Services Reserach, 46(5), 1517-1533.
Reference:	Stock, R., Scott, J., & Gurtel, S. (2009). Using an Electronic Prescribing System to Ensure Accurate Medication Lists in a Large Multidisciplinary Medical Group. The Joint Commission Journal on Quality and Patient Safety; 35(5), 271-277.
Reference:	Tache, S.V., Sonnichsen, A., & Ashcroft, D.M. (2011). Prevalence of Adverse Drug Events in Ambulatory Care: A Systematic Review. The Annals of Pharmacotherapy, 45(7-8), 977-989. doi: 10.1345/aph.1P627.
Reference:	The Joint Commission (2015). Ambulatory Care National Patient Safety Goals. Retrieved from http://www.jointcommission.org/assets/1/6/2015_NPSG_AHC1.PDF
Reference:	Weeks, D.L., Corbette, C.F., Stream, G. (2010). Beliefs of Ambulatory Care Physicians about Accuracy of Patient Medication Records and Technology-Enhanced Solutions to Improve Accuracy. Journal for Healthcare Quality; 32(5), 12-21.

Definition:	Current Medications: Medications the patient is presently taking including all prescriptions, over-the-counters, herbals and vitamin/mineral/dietary (nutritional) supplements with each medication's name, dosage, frequency and administered route. Route: Documentation of the way the medication enters the body (some examples include but are not limited to: oral, sublingual, subcutaneous injections, and/or topical).
Guidance:	This measure is to be reported for every encounter during the measurement period. Eligible professionals or eligible clinicians reporting this measure may document medication information received from the patient, authorized representative(s), caregiver(s) or other available healthcare resources. This list must include all prescriptions, over-the-counter (OTC) products, herbals, vitamins, minerals, dietary (nutritional) supplements AND must contain the medications' name, dosage, frequency and route of administration. This measure should also be reported if the eligible professional or eligible clinician documented the patient is not currently taking any medications. By reporting the action described in this measure, the provider attests to having documented a list of current medications utilizing all immediate resources available at the time of the encounter.
Measure Population:	
Measure Observations:	
Supplemental Data Elements:	For every patient evaluated by this measure also identify payer, race, ethnicity and sex